BASELINE → TREATMENT → FOLLOW-UP

Abemaciclib: baseline checks and the first four months

NCCP 00619 specifies baseline ECG as clinically indicated. It does not mandate the ribociclib Cycle 1 Day 14 ECG schedule. Abemaciclib is continuous twice daily: there is no routine seven-day break.

Check	Before first dose	First 2 months	Next 2 months	After month 4
FBC / renal / liver / bone profile	Baseline blood tests	Every 2 weeks	Monthly	As clinically indicated
ECG · heart rhythm	As clinically indicated	Repeat if clinically indicated	Repeat if clinically indicated	Symptoms / risk guide testing
Diarrhoea / clots / breathing	Agree action plan and assess clot history	Early symptom review; do not wait for bloods	Review symptoms and adherence	Continue prompt symptom reporting
Bone health / endocrine partner	Fracture-risk assessment; DEXA if indicated	Endocrine / ovarian-suppression plan continues	Manage symptoms and bone risk	Repeat DEXA according to risk / treatment

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00619 v4a · official regimen · NICE NG101 · bone health and follow-up

THE WHOLE TREATMENT JOURNEY

Adjuvant abemaciclib: two years within a longer plan

Adjuvant · high-risk node-positive HR-positive / HER2-negative early breast cancer

Abemaciclib is taken with endocrine treatment after surgery and other indicated local/systemic treatment. Abemaciclib is planned for two years; endocrine treatment commonly continues beyond that.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Earlier treatment	Before abemaciclib	Months 1–2	Months 3–4	Months 5–24	After abemaciclib
Surgery / chemotherapy NCCP 00619 · EARLY	Lumpectomy OR mastectomy; chemotherapy if indicated	Confirm recovery and eligibility	No routine additional chemotherapy in this regimen	Review symptoms and adherence	Continue recovery / rehabilitation	Long-term support
Radiotherapy NCCP 00619 · EARLY	After surgery and chemotherapy when indicated	Confirm completion and start plan with team	Any overlap requires an explicit specialist plan	Review late skin / breast symptoms	Ongoing symptom review	Ongoing symptom review
Abemaciclib NCCP 00619	Discuss suitability	Check medicines and provide diarrhoea plan	Twice daily continuously	Continue without a planned week off	Continue to 2 years if appropriate	Stop at planned completion or earlier if needed
Endocrine treatment NCCP 00619 · EARLY	May already have started	Confirm chosen drug; ovarian suppression if needed	Continue as prescribed	Continue as prescribed	Continue as prescribed	Usually continues for years; review duration
Blood tests NCCP 00619	Review previous results	Blood count, kidney/liver and bone profile	Every 2 weeks	Monthly	As clinically indicated	As needed for ongoing treatment
Bone health BONE · NOGG	DEXA may precede abemaciclib	Review fracture risk and prior DEXA	Bone protection if indicated	Review exercise, nutrition and symptoms	Repeat DEXA by risk, often considered at 12–24 months	Continue endocrine-related bone plan
Cancer / heart tests NCCP 00619 · EARLY · ESC	Record baseline cancer and cardiac history	No routine echo solely for abemaciclib	Tests for symptoms or prior-treatment risks	Remaining-breast surveillance continues	Annual mammography; no routine CT/markers if well	Continue agreed surveillance

What each check is for

1 Early blood monitoring

Baseline; every 2 weeks for 2 months; monthly for the next 2 months

Checks blood counts and kidney/liver/bone-profile results.

Ask / act: Confirm blood-test appointments even though the medicine is taken at home.

Sources: NCCP 00619

2 Early diarrhoea support

From the first loose stool and throughout treatment

Diarrhoea can develop early and cause dehydration.

Ask / act: Use the prescribed rescue plan and contact the team; do not wait for the next review.

Sources: NCCP 00619 · ABEMA

3 Bone density and fracture risk

At AI/OFS initiation and then by risk

Assess fracture risk and whether a baseline DEXA bone-density scan is needed when starting an aromatase inhibitor or ovarian suppression/treatment-induced menopause. It may already have been done before this regimen.

Ask / act: Ask when bone health was last assessed and whether a repeat scan or bone medicine is needed.

Sources: BONE · NOGG

4 Breast surveillance

After early breast treatment

Annual mammography of remaining breast tissue after treatment; the team sets the first date. The mastectomy side is not routinely mammographed. New symptoms need assessment between visits.

Ask / act: Report symptoms promptly; a normal scheduled mammogram does not explain every new symptom.

Sources: EARLY · BONE

5

Two-year transition

At planned abemaciclib completion

The CDK4/6 drug ends while endocrine treatment may continue.

Ask / act: Ask for the continuing endocrine, bone-health and follow-up plan.

Sources: NCCP 00619 · EARLY

6

Baseline ECG

Before starting, as clinically indicated

Cardiac symptoms, history or other medicines may make an ECG appropriate.

Ask / act: Ask whether a baseline ECG is needed for your situation.

Sources: NCCP 00619

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.